

Guideline Title: Abdominal and Genitourinary Trauma

Guideline Number: GUID-3203-T001

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- I. **PURPOSE:** To recognize the patient who has suffered abdominal and genitourinary (GU) traumatic injury, stabilize the patient by controlling hemorrhage, provide patient with adequate airway, perfusion, and pain management, prevent hypothermia, and transport the patient to a facility able to immediately provide the patient with definitive trauma management.
- II. **DEFINITIONS:**
 - A. SBP – Systolic Blood Pressure
- III. **DEPARTMENT GUIDELINE:**
 - A. Obtain from report: mechanism of injury, vehicle damage to steering wheel/column, use of seat belt, intrusion into compartment, ejection of patients, height of fall, speed at which was struck.
 - B. Establish care as defined by *GUID3203-G002 General Patient Care* and *GUID3203-T008 Multi-system Trauma*
 - C. Recognize that injuries between the nipple and perineum need to be considered as intraperitoneal and injuries between the umbilicus and nipple line may involve the thoracic cavity.
 - D. Assess for and control any external hemorrhage.
 - E. Assess and monitor for signs of shock, altered mentation, a decreased blood pressure or increased heart rate, and or signs of peripheral vascular constriction.
 - F. Two large bore IV's (size appropriate for patient age) should be inserted.
 - G. Adult patients with shock
 - i. Administer warmed 0.9% NS or Lactated Ringers to maintain SBP 80-90 mmHg
 1. Consider permissive hypotension if no traumatic brain injury is suspected or present.
 - ii. For suspected hemorrhagic shock, consider blood product transfusion per *GUID3203-PR025 - Emergency Blood Product Administration* or *GUID3203-PR002 Blood Product Administration*.
 - iii. Consider use of TXA per *GUID3203-T016 - Tranexamic Acid for Severe Hemorrhage*.
 - H. Pediatric patients with shock
 - i. Administer 20 ml/kg warmed 0.9% NS or Lactated Ringers, may repeat once

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- ii. For suspected hemorrhagic shock, consider blood product transfusion per *GUID3203-PR025 - Emergency Blood Product Administration* or *GUID3203-PR002 Blood Product Administration*.
 - iii. Permissive hypotension is contraindicated in pediatric patients.
- I. Assess for
 - i. Any entry and exit wounds
 - ii. Varying degrees of abdominal pain during a rapid but all-inclusive abdominal examination
 - iii. Signs of internal hemorrhage
 - 1. Distended abdomen
 - 2. Kehr's Sign (sudden, sharp acute sharp pain at the tip of the shoulder) can be indicative of left hemidiaphragm irritation)
 - 3. Cullen's sign (ecchymotic discoloration around the umbilicus) can indicate intra-abdominal or retroperitoneal hemorrhage
 - 4. Grey Turner's sign (Flank ecchymosis may signify retroperitoneal hematoma formation from trauma to the kidneys, major blood vessels, or other organs)
 - iv. Assess for evidence if myoglobinuria and monitor urinary output if indwelling urinary catheter is present from sending facility.
- J. If patient has an unstable pelvis or a suspected open book pelvic fracture, a commercial pelvic binder or tied sheet should be used to stabilize the pelvis per *GUID3203-PR027 Pelvic Circumferential Compression Binder*.
 - i. Avoid repeated assessments by manual compression of an unstable pelvis.
- K. Place patient in full spinal precautions.
- L. Repeat assessments of abdominal girth, distention, and discoloration.
- M. Stabilize and do not remove penetrating objects. If transport cannot safely be accomplished in an aircraft due to a penetrating object, alternate mode of transport must be initiated.
- N. Treat pain and/or anxiety per *GUID3203-G005 - Analgesia and Sedation Management*.
- O. Treat nausea/vomiting per *GUID3203-G004 - Motion Sickness/Nausea*.
- P. Gastric tube placement is indicated for all intubated per *GUID3203-PR010 - Gastric Tube Insertion*.
 - i. OG placement is preferable.
 - ii. Patients with diaphragmatic rupture, GI tract injuries, pregnancy, suspected intestinal injury, gastric distention, or potential for aspiration should also have a gastric tube placed.
- Q. Apply saline dressings to any eviscerated bowel or dry dressings to any open wounds.
- R. Protect patient from hypothermia using passive and active rewarming.
- IV. **DOCUMENTATION:**
 - A. EMS charts
- V. **REFERENCES:**